

Target Chiropractor ICP + Ad Copy

Who we're going after in Virginia, the pains that move him, and compliance-clean ad copy ready to run.

The Ideal Partner — "The Town's One Guy"

Who he is: the single dominant, longest-established chiropractor in a Virginia town of ~150K — the name everybody refers to, with 20 years of community trust. His identity is "the doctor," not "a business owner"; the practice is his life's work. Skews strongly male, core age **45-60** (40+ leading edge). Married, college-age kids, underfunded retirement. High income while working (\$200K-\$500K+ take-home) but net worth trapped in a practice that won't sell for much.

The practice: single-owner / solo-DC (or owner + 1 associate), independent — not a franchise, not PE-owned. 10-25+ years at one location. ~\$800K-\$1.5M gross (sweet spot ~\$1.3M). **25+ new patients/mo** on a 2-year baseline (40+ is strong). Strong retention. Rehab + PT + chiropractic focus. Accepts insurance AND Medicare (altruistic). Often owns or long-leases a building with **spare square footage**. Small team (3-10).

The wound: on a standalone sale these practices trade at ~1.65× SDE — a goodwill-heavy \$1.3M practice can list for ~\$350K. Twenty years of relationships count for zero. He's trapped: great income, almost nothing to sell, no exit, and the clinic dies with him.

What he wants: more new patients (without spending his own time or money), to serve patients better (advanced diagnostics), more income now, and — once he sees it's possible — a real exit. Above all: keep practicing exactly as he does, same name on the door, and NOT have to run a medical system.

Positioning rules (non-negotiable): brand strictly as **PROMEDVA / corporate** — never founder-led (competitors won't sign if they think a rival DC is behind it). **Hide the rollup** — never expose platform multiples or acquisition arbitrage in marketing. Virginia-only; the focus-city cluster (Waynesboro · Staunton · Harrisonburg · Lynchburg) first. Cold copy leads with identity + the valuation shock + income-without-change; the deepest exit/goodwill gut-punch converts best in warm/retargeting.

Top Pains (in Dr. Wagner's voice)

- **Goodwill = \$0 on a sale.** A \$1.3M-revenue practice may sell for only ~\$350K — 20 years of relationships count for zero.
- **Trapped, no exit.** "All those clinics die with the doctor — there's no transfer of equity."
- **The body is the business.** "Chiropractors will die on the table because they don't know how else to provide their same lifestyle."
- **The squeeze.** Waking up to college costs + an underfunded retirement.
- **Can't grow / can't run it.** Most "act like they're broke"; can't implement an advanced medical system themselves ("they'd screw it up").
- **Medicare drains them.** A money-losing line item carried out of conscience.

Ad Angles — Ranked (judged for resonance + compliance)

1. **Retirement + College Squeeze (8/10 — LEAD):** "You can't adjust patients forever — fund the kids' college and your retirement."
2. **Goodwill (7/10):** "Your 20 years of patient relationships have real value a standard sale throws away."
3. **Better Care + You Get Paid (7/10):** advanced diagnostics that validate the care you already give — and finally get paid for it. (Best for warm/credibility.)
4. **The Trap / No Exit (7/10 — RESERVE for warm + retargeting):** "Right now your clinic dies with you. We change that." Too heavy for a first cold touch.

The pure "rent / add ~\$250K in practice growth" angle under-performs as a cold ad — its best elements (the income number + "it isn't your job to run") are folded into the angles above. Use it as a *landing-page* headline, not a cold hook.

Ready-to-Run Ad Copy

Angle 1 • Retirement + College Squeeze (lead)

Hook: You can't adjust patients forever. So why is your retirement plan "keep working"?

Here's the quiet math no one runs for you: your body is the business. The day it slows down, so does the income. Tuition bills don't slow down. Neither does retirement. PROMEDVA partners with Virginia's leading chiropractors to add a cash-services line inside your existing clinic — we bring the medical team, the diagnostics, and the capital. You provide the space you already have and keep practicing exactly as you do today, same name on the door. The result could be new monthly cash flow on top of what you earn now (illustrative; depends on your market — no guarantees), plus a real path to finally getting paid for the goodwill a standard sale ignores. Established Virginia practices only.

Headline: Stop letting "keep working" be your retirement plan. · **CTA:** See If You Qualify

Hook: College tuition. Retirement. And a practice that only pays you while you're standing in it.

If you stopped working next month, what would your practice actually be worth? For most established Virginia chiropractors the honest answer stings. A standalone sale prices last year's numbers and treats 20 years of patient relationships as zero — a great income you can't sell. PROMEDVA was built to fix that: we lease space inside your clinic, run our own licensed medical team for advanced cash services, and pay you rent plus a share of that cash-service revenue (illustrative — no income guarantees). You don't learn a new system or run anything medical. Established Virginia practices only — availability limited.

Headline: A great income you can't sell. Let's change that. · **CTA:** See If You Qualify

Cold email — subject: "The retirement math most VA chiropractors don't run until it's late" | **Video hook:** "You can't adjust patients forever — so why is 'keep working' your retirement plan? Everybody in town refers to you, and your practice still might sell for almost nothing. Virginia chiropractors: there's a better exit."

Angle 2 · Goodwill

Hook: A broker will tell you your practice is worth a few hundred grand. They're valuing 20 years of your life like a used car.

When you sell the standard way, your practice gets valued on last year's net income — and almost nothing else. Two decades of patient relationships. The reputation that makes everyone refer to you. On paper, all of it counts as zero. PROMEDVA exists to fix that. We partner with the leading chiropractor in a market, lease space for our own medical team to deliver advanced cash services, and pay you monthly rent plus a share of cash-service revenue (illustrative — depends on your market, no guarantees). You keep practicing exactly as you do today. And once we've proven the fit, there's a path designed to finally pay you for the goodwill a normal sale ignores. Virginia only — one leading practice per market.

Headline: Your practice could be worth far more than they're telling you. · **CTA:** See If You Qualify

Hook: Twenty years of patient relationships, community reputation, trust. On a standard sale, all of it is worth \$0.

Most Virginia chiropractors don't find this out until they try to sell — and by then it's too late. The goodwill you've built has no line item in a standalone valuation; the practice gets priced on net income and the offer lands far below what you assumed. We think that's backwards. PROMEDVA brings our own medical team, diagnostics, and capital to deliver advanced cash services from space we lease inside your clinic. You earn monthly rent plus a share of cash-service revenue (illustrative — varies by market, no guarantees), with zero change to how you treat patients. Then, after we prove the fit, a path designed to monetize the goodwill a normal sale ignores.

Headline: The value in your practice that no broker counts. · **CTA:** See If You Qualify

Cold email — subject: "What your 20 years are actually worth (most Virginia DCs find out too late)"

Angle 3 · Better Care + You Get Paid (altruism-first; strong for warm)

Hook: You've heard "add a medical service line" before. Here's why this one isn't on your plate.

Every "add medical services" pitch fails for the same reason: it dumps a complicated system on the doctor — new billing, new protocols, new headaches — so it sits unused. PROMEDVA is built the opposite way. We run our OWN licensed medical team, end to end, inside space we lease from your clinic. You don't run it, staff it, or fund it. What you get is a new cash-services line operating alongside you — potentially a meaningful five-figure-plus monthly opportunity in our partner clinics (illustrative only — no income guaranteed) — added on top of what you already make. Your insurance and Medicare patients stay 100% yours and unchanged. Virginia-first — one leading practice per market.

Headline: The medical line that isn't your job to run. · **CTA:** See If You Qualify

Cold email — subject: "The diagnostics your treatments have earned" | **Video hook:** "You've spent twenty years building judgment most doctors never develop. Now imagine advanced diagnostics to document every patient's progress — brought into your clinic, run by someone else, paying YOU. Virginia chiropractors only."

Angle 4 · The Trap / No Exit — WARM / RETARGETING ONLY

Do not run cold. Serve only to email openers, 50%+ video watchers, and landing-page visitors — it converts hardest there but is too heavy as a first touch.

Hook: "There's no transfer of equity. The clinic just dies with the doctor." If that sentence stings, read this.

When you're ready to step back, a standard sale pays you for last year's numbers — not for the reputation and relationships that ARE the practice. The equity you assume is there mostly isn't; it dies with you. PROMEDVA changes the ending. We partner with leading Virginia chiropractors as a corporate medical platform — not a competitor after your patients. You keep practicing exactly as you do now. We bring the medical team and capital, lease space in your clinic, and pay you monthly for advanced cash services (illustrative — no income guarantees). Then, only after we've proven the fit together, a path designed to honor the goodwill a fire-sale ignores. Virginia only.

Headline: Stop letting your life's work transfer nothing to you. · **CTA:** See If You Qualify

Compliance Checklist (every asset)

- **Cash-services only** — income tied to rent and/or a cash-pay medical-services line; insurance/Medicare walled off ("stays 100% yours"). No fee-split or kickback implication.
- **No income guarantees** — every figure hedged (illustrative / could / potential / depends on market).
- **No medical-efficacy claims** — diagnostics "document and measure progress," never "prove treatments work."
- **Virginia-only** stated in every ad, email, and video hook.

- **No gender/age language in copy** — male/45-60 skew achieved via age + interest targeting only.
- **Hide-rollup** — no platform multiple or arbitrage anywhere in marketing.
- **Corporate PROMEDVA branding**, never Wagner-led; "not a competing clinic" reassurance included.
- **VA counsel sign-off** on the rent + cash-pay-services structure before scaling spend (Dr. Wagner's own counsel).

Virginia Targeting Plan (in sequence)

1. **Owned 24K-DC email list, VA segment** (start here — highest intent, lowest cost): filter to VA DCs, prioritize the focus-city cluster; 4-5 touch educational sequence; PROMEDVA / corporate branding.
2. **Meta lead-gen + retargeting**: VA-only, age 45-65 + chiropractic-ownership/practice-management interests; lead with Retirement/College, then Goodwill, then Better Care; serve Trap/No-Exit only to warm pools.
3. **Google Search + YouTube** (geo-fenced VA): "sell chiropractic practice Virginia," "chiropractic practice valuation," "add medical services to chiropractic practice." Land on the "Value My Clinic" tool + qualifying intake.
4. **Chiropractic associations + events** (UVCA, VA society): sponsor/speak/newsletters for credibility and warm reach.
5. **Warm/known practices in the focus cities**: handled 1:1, corporate-PROMEDVA branded (principal off all marketing).

PROMEDVA · Confidential. Ad copy is compliance-scrubbed (cash-only, no income guarantees, no medical-efficacy claims, VA-only, no gender/age language, rollup hidden). Final structure subject to Virginia healthcare counsel. Run within ad-platform policy (confirm no Special Ad Category restriction applies).